NURSING MANAGEMENT FUNCTION: CONTROLLING

CONTROLLING

→ Includes **ASSESSING and REGULATING performance** in accordance with the plans that have been adopted, the instructions issued, and the principles established.

→ **Provides information** about how well processes & people function so they can be motivated to perform better in the future

→ **Types of Organizational Controls:**

1. **Feedforward control**: prevents problems by monitoring inputs and anticipating issues before they affect the system
2. **Concurrent control**: monitors ongoing activities in real-time, ensuring they conform to standards as they happened
3. **Feedback control**: occurs after activities are completed, evaluating outputs and using past performance to improve future results.

REASONS FOR CONDUCTING EVALUATION

✪ Ensures that **quality nursing care** is provided.
✪ Allows for the **setting of sensible objectives** and ensures compliance with them.
✪ **Provides standards** for establishing comparisons.
✪ **Promotes visibility** and a means for employees to monitor their own performance.
✪ Highlights problems related to quality care and **determines the areas that require priority** attention.
✪ Provides an indication of the costs of poor quality.
✪ **Justifies the use of resources**.
✪ **Provides feedback** for improvement.

PRINCIPLES OF EVALUATION:

✪ Must be **based on the BEHAVIORAL STANDARDS** of performance.
✪ There should be **enough time** to observe employee's behavior.
✪ Employee should be given a copy of the job description, performance standards & evaluation form.
✪ Performance appraisal should include **satisfactory & unsatisfactory results**
✪ Areas needing improvement must be prioritized
✪ **Evaluation conference** should be scheduled and conducted at a convenient time for the rater and employee
✪ Evaluation report and conference **should be structured**

CHARACTERISTICS OF AN EVALUATION TOOL	
Objectivity	Tool is free from bias
Reliability	Tool is accurate
Validity	Tool is relevant
Sensitivity	Tool can measure fine lines of differences among the criteria being measured

BASIC COMPONENTS OF THE CONTROL PROCESS

A. Establishment of Standards, Objectives & Methods for Measuring Performance

- **Types of Performance Standards**
  1. **Structure standards**: focus on the structure or management system used by the agency to deliver care
  2. **Process standards**: action of the nurse relative to the nursing process
  3. **Outcome standards**: designed to measure the results of care provided.
- **Methods for Measuring Nursing Care**
  - Task Analysis
  - Quality Control

B. Measuring Actual Performance

- An **ongoing, repetitive process** with the actual frequency dependent on the type of activity being measured
- Maybe scheduled in advance/maybe done at periodic but unannounced intervals

C. Comparing Results of Performance with Standards & Objectives

- Performance should match with standards & objectives

D. Reinforcing Strengths & Taking Corrective Action

- Corrective actions are applied to improve performance

PERFORMANCE APPRAISAL

→ Employee's **performance is evaluated** against standards.
- Reflects **how well** the nursing personnel have performed during a specific period of time.

→ **Purpose**

✪ Determine salary standards
✪ Select qualified individuals for **promotion**
✪ **Identify unsatisfactory employees** for demotion
✪ Make inventories of talents
✪ **Determine training needs** of employees
✪ Improve the performance of work groups
✪ Improve communication
✪ **Establish standards** of supervisory performance
✪ Discover the aspirations of employees
✪ Provide "employee recognition" for accomplishments
✪ Inform employees "**where they stand**."

FACTORS OF EFFECTIVE PERFORMANCE APPRAISAL:

✪ Appraisal should be **based on a standard**
✪ **Employee should have input** into the development of the standard.
✪ Employee **must know the standard in advance**
✪ Employee must know the sources of data gathered for the appraisal.
✪ Appraiser should be someone who/whom:
  - Has **observed employee's work**.
  - The **employee trusts** and respects.

METHODS OF MEASURING PERFORMANCE

1. Informal appraisal

- May consist of "**INCIDENTAL OBSERVATION**" of performance **while the worker is engaged** in performing nursing care
  - **Example**: Nurse manager notices a staff nurse not following hand hygiene protocol before patient care.

2. Formal appraisal

- Accomplished regularly and methodically by collecting **objective facts that can demonstrate** the difference between what is expected and what was done
  - **Example**: Chart audits to check accuracy and completeness of documentation.

PERFORMANCE APPRAISAL TOOLS

1. **ESSAY**
   - Referred to as the **free-form review**
   - The **appraiser describes in narrative form** an employee's strengths & areas where improvement is needed
2. **RANKING**
   - The appraiser **ranks the employees** according to how he fared with co-workers with respect to qualifications.
3. **CHECKLISTS**
   - A **compilation** of all nursing performances expected of a worker.
   - **Types:**
     - **Weighted scale**: composed of **many behavioral statements** that represent desirable job behaviors.
     - **Forced checklist**: requires the supervisor to select an **undesirable and a desirable behavior** for each employee.
     - **Simple checklist**: composed of **numerous words or phrases describing various employee** behaviors or traits. The rater will check all those that describe the employee on each checklist.
4. **RATING SCALE**
   - Includes a **series of items representing the different tasks** or activities in the nurse's job description or the absence or presence of desired behaviors and the extent to which these are possessed.
5. **FORCED-CHOICE COMPARISON**
   - The evaluator will choose the **statement that best/least describes the nurse being evaluated**
   - The items are grouped that the evaluator is forced to choose from favorable as well as unfavorable statements
6. **ANECDOTAL RECORDING**
   - Describes the **nurse's experience with a group or a person**, or in validating technical skills and interpersonal relationships.
7. **PEER REVIEW**
   - May be **done by peers (employees of the same profession, rank and setting)** evaluating another's job performance against accepted standards.

COMMON PITFALLS OF PERFORMANCE APPRAISAL:

✪ **HALO EFFECT**
- When the appraiser lets one or two **positive aspects of the behavior of the employee influence all** other aspects of the employees' performance.

✪ **HORNS EFFECT**
- When the appraiser allows some **negative aspects** of the employee's performance influence the assessment to such extent that other levels of job performance are not accurately recorded.

✪ **CENTRAL TENDENCY TRAP**
- The **appraiser is hesitant to risk** true assessment and therefore rates all employees as average.

QUALITY ASSURANCE

→ **Evaluation that is applied to the health care system** and the provision of health care services by health workers.

A. Quality Improvement Program
- A **continuous on-going measurement and evaluation process** that includes structure, process, and outcome
- **Example**: Monitoring hand hygiene compliance and implementing strategies to reduce hospital-acquired infections.

B. Benchmarking
- Process of measuring products and services against those of best-performing organizations.
- **Example**: A hospital compares its patient satisfaction scores with top-rated institutions to improve its service delivery.

C. Quality of care
- The degree to which health services for individuals & population increase the likelihood of desired health outcomes
- **Example**: Measuring effectiveness of a stroke care protocol based on patient recovery rates.

D. Nursing Audit
- **Documentation of the quality of nursing care** in relation to the standards established by the nursing department
- **Example**: Auditing patient charts to check if pain assessments are consistently documented every shift.

METHODS OF QUALITY ASSURANCE

1. **Patient Care Audits**
   - **Concurrent**: Patient care is observed and evaluated. Patients' charts are reviewed while the patients are still confined in the hospital
     - **Example**: Checking IV site care and charting while patient is confined.
   - **Retrospective**: Patient care is evaluated thru review of discharged patient's chart & questionnaires sent to or interviews on discharged patients
     - **Example**: Evaluating completeness of discharge instructions in patient charts.
2. **Peer Review**
   - A **collegial evaluation of the performance** done to promote excellence in practice
     - **Example**: A wound care team reviews another nurse's dressing technique for adherence to sterile procedure.
3. **Quality Circles**
   - **Group of workers doing similar work** who meet regularly, voluntarily on normal working time under the leadership of their supervisor.
     - **Example**: Staff nurses meet monthly to suggest ways to reduce medication errors in their unit.
4. **Structure Audit**
   - **Monitors the structure or setting** in which patient care occurs such as finances, nursing service structure, medical records, environmental structure.
     - **Example**: Checking if there are enough IV pumps, safe staffing ratios, and proper documentation systems.

5. **Process audit**
   - Measure how nursing care is provided or how the care was carried out
     Used to **provide a tool to assess deviations** from accepted best practice process standards
     - **Example**: Reviewing whether nurses followed the 5 Rights of Medication Administration.
6. **Outcome Audit**
   - Determines what results if any, occurred as a result of specific nursing intervention for patients
   - The **most valid indicators** of quality care
     - **Example**: Measuring patient fall rates, infection rates after interventions.

DISCIPLINE

→ A **constructive and effective** means by which **employees take personal responsibility for their own performance** and behavior.

FACTORS THAT INFLUENCE SELF-DISCIPLINE:
✪ A **strong commitment** to the vision, philosophy, goals and objectives of the institution.
✪ Laws that govern the practice of professionals and their **respective codes of conduct**.
✪ Understanding the **rules and regulations** of the agency
✪ An **atmosphere of mutual trust** and confidence
✪ Pressure from peers and organization.

DISCIPLINARY APPROACH

→ A **sound disciplinary program** must be tailored to the objectives of the institution.

→ **Should include the following:**
- Set of disciplinary policies and procedures
- A uniform application of the rules
- Disciplinary committee
- Orientation program for all new employees

→ **Successful Implementation of a disciplinary action:**
- **Promptness**: Action is taken immediately after the violation
  - **Example**: Nurse manager addresses a staff nurse who repeatedly arrives late on the same shift, not weeks later.
- **Fairness**: Discipline matches severity of the offense.
  - **Example**: Minor documentation errors = coaching, not suspension.
- **Impartiality**: Rules applied consistently to all staff, without favoritism
  - **Example**: Both senior and new nurses receive the same consequence for medication errors.
- **Non-punitiveness**: Aim is correction, not punishment
  - **Example**: Nurse manager provides retraining on IV insertion instead of only issuing a warning for a failed attempt.
- **Advance warning**: Staff are informed of rules and possible consequences beforehand
  - **Example**: Hospital handbook and orientation clearly state attendance policies.
- **Follow-through**: Management enforces rules consistently and doesn't ignore violations
  - **Example**: Continuous tardiness leads to progressive discipline (verbal → written → formal action).

DISCIPLINARY ACTION

1. **COUNSELING AND ORAL WARNING**
   - **The employee is counseled regarding:**
     - Expectations of improved performance
     - Ways of correcting the problem
     - Warning that a repetition of the same offense may warrant further disciplinary action.
   - **Example**: A nurse is verbally counseled after being late for duty multiple times in one week.
2. **WRITTEN WARNING**
   - This includes the statement of the problem, identification of the rule which was violated, consequences of continued deviant behavior, and the employee's commitment to take corrective action, and any follow-up action to be taken
   - **Example**: Nurse receives a written warning for incomplete charting after reminders and oral warning.
3. **SUSPENSION**
   - Suspension over a **minor violation** is given after evidence of oral and written warnings
     - Suspension rather than dismissal is applied when management feels that the employee can still be rehabilitated
   - **Example**: Nurse suspended for 3 days after repeated medication errors despite prior counseling and written warning.
4. **DISMISSAL**
   - Invoked only when all disciplinary efforts have failed
     - The **disciplinary committee** should be very sure that the cause for dismissal conforms with the criteria of a major discipline violation as contained in the policy manual
   - **Example**: Nurse dismissed for theft of hospital property after repeated disciplinary actions.